ASTER CMI MULTISPECIALTY HOSPITAL

New Airport Road, NH 44, Hebbal, Bengaluru 560092 | Joint Commission International (JCI) Accredited | SYNTHETIC DEMO DATA

Patient Name: Priya Venkat	**UHID / IP No:** P1002 / IP-2026-904
Age / Gender: 34 Yrs / Female	**Admission Date:** 12-Feb-2026 | **Discharge:** 14-Feb-2026
Primary Consultant: Dr. Meera Nambiar (MD Pulm)	**Allergies: ASPIRIN & NSAIDs (BRONCHOSPASM)**

DISCHARGE CLINICAL SUMMARY & DIAGNOSES

1. Acute Exacerbation of Bronchial Asthma (Moderate Persistent, ICD-10: J45.40)
2. Primary Hypothyroidism (E03.9) on hormone replacement therapy

HOSPITAL COURSE & LABORATORY HIGHLIGHTS

Laboratory Test / Parameter	Discharge Value	Reference Range	Clinical Interpretation
Thyroid Stimulating Hormone (TSH)	**7.82 mIU/L**	0.40 - 4.20 mIU/L	**Elevated; dose titrated to 75 mcg**
Free Thyroxine (FT4)	0.88 ng/dL	0.80 - 1.80 ng/dL	Low Normal
Absolute Eosinophil Count (AEC)	**620 cells/mcL**	20 - 500 cells/mcL	**Elevated (Allergic phenotype)**
Serum IgE Total	**410 IU/mL**	< 100 IU/mL	**High Atopic Marker**
SpO2 (Room Air)	98%	> 95%	Adequate Oxygenation

DISCHARGE MEDICATIONS (Rx)

#	Drug Name & Dosage	Administration Schedule	Special Instructions
1	Budesonide + Formoterol Turbuhaler (160/4.5 mcg)	2 Puffs Twice Daily (BD)	Mouth rinse after inhalation
2	Tab. Montelukast 10 mg	1 Tab OD at bedtime (HS)	Leukotriene receptor antagonist
3	**Tab. Levothyroxine Sodium 75 mcg**	1 Tab OD early morning	Strictly empty stomach 45m before breakfast

MANDATORY SAFETY ADVISORY:

• **STRICT CONTRAINDICATION: NSAIDs & ASPIRIN.** Even small doses of Ibuprofen, Naproxen, or Diclofenac trigger life-threatening acute asthma exacerbations (AERD).
• For analgesia, use Paracetamol (Acetaminophen) with physician guidance.
• Follow up with Pulmonology & Endocrinology in 6 weeks with repeat TSH.

Dr. Meera Nambiar, MD, DNB (Pulmonology) | KMC Reg #44102 | Aster CMI Hospital

SYNTHETIC DEMO DATA - Complies with hackathon privacy and evaluation standards.